



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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March 18, 2026

VIA ELECTRONIC MAIL

Evan Escobar
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Valley Children's Hospital
Block Transfer Filing No. 2026-HOSP-293
Filed on March 17, 2026

Dear Mr. Evan Escobar:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Valley Children's Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Regarding the Enrollee Transfer Notice submitted for this Filing, the following information was found missing. Please resubmit the Enrollee Transfer Notice for the Department's review to include the following:
 - a. Statement and/or attachment informing enrollees that they may contact the Plan for language assistance. This statement must include the contact information for these services written in the identified top 15 threshold languages spoken by limited-English-proficient individuals in California as determined by the DHCS.

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Contact the DMHC Help Center at 1-888-466-2219 or www.DMHC.ca.gov

Plan Response: Please see the attached Babel sheet that is included with the Enrollee Transfer Notice.



2. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval

Plan Response: The Plan confirms that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval

3. Please confirm whether the Plan has an internal deadline to start the Enrollee Transfer Notice. If yes, please share that date with the Department.

Plan Response: No internal deadline for Enrollee Transfer Notices related to Hospital terminations.

4. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

Plan Response: The hospital is not part of a health system termination.

5. On the Block Transfer Filing form, in the Filing Info section, the Plan indicated there were zero (0) bed days utilized and zero (0) admits at the Terminating Hospital for the most recently completed Calendar Year. However, in the Enrollee Access section of the Block Transfer Filing form, Plan indicated that **LaSalle Medical Associates** has 91,823 assigned enrollees, utilized 79 bed days, and 35 non-ER admits. Please confirm the following:

- a. The total number of affected enrollees, **In progress**
- b. The total number of bed days used, **In progress**
- c. The number of non-ER admits, **In progress**
- d. Whether LaSalle Medical Associates has routine access to the Terminating Hospital **LaSalle Medical Associates is being evaluated to confirm whether it has routine access to Valley Children's Hospital.**
- e. If the pending contract termination impacts 50,000 or more enrollees, please submit the relevant portions of the Provider Contract(s) that relate to continuity of care and transition of care.

Plan Response: The Plan will submit the relevant provider contract provisions related to continuity of care and transition of care

6. Please confirm whether the contract termination between the Plan and Valley Children's Hospital impacts Point of Service (POS) enrollees **no impact**
7. Please indicate whether the Terminating Hospital is or provides any of the following:
 - a. Tertiary care, quaternary care, cancer, burn or other high-level specialty services, **Yes. Valley Children's Hospital provides high-level pediatric specialty services, including Level IV NICU/Regional NICU services, PICU services, Level II Pediatric Trauma services, and pediatric cancer services.**
 - b. Transplant services, and if so,
 - i. What types of transplants **bone marrow/stem cell**
 - ii. Whether the contract termination includes the transplant contract, **No**
 - iii. The termination date of the transplant contract if separate from the hospital contract. **Not applicable**
 - c. Recognized or designated as a hospital that provides other high-level specialty services, such as a Center of Excellence for Bariatric Surgery, Blue Distinction Center for Spine Surgery, etc., and if so, **Yes. Valley Children's Hospital is recognized for high-level pediatric specialty services.**
 - i. Identify the distinguished services. **Distinguished services include neonatal intensive care (Level IV NICU), pediatric trauma, pediatric epilepsy, extracorporeal life support (ECMO), pediatric inpatient rehabilitation, cleft/craniofacial services, and antimicrobial stewardship.**
8. For each service available at the Terminating Hospital from Comment 7.a, 7.b, and 7.c, please confirm which Alternate Hospital(s) provides each service(s).

Plan Response: The Plan has evaluated the services identified in Question 7 and confirms that alternate in-network hospitals in the Central Valley provide general acute care and limited pediatric services; however, they do not provide equivalent pediatric tertiary and quaternary services at the same scope or designation as Valley Children's Hospital. Specifically: Level IV NICU/Regional NICU, PICU, pediatric trauma, ECMO, pediatric cancer (comprehensive), and advanced pediatric subspecialty services: Not available at equivalent level at alternate hospitals. Bone marrow/stem cell transplant (pediatric): Not available at identified alternate in-network hospitals. Other recognized specialty services (e.g., pediatric epilepsy, pediatric rehabilitation, cleft/craniofacial, antimicrobial stewardship): Not available at equivalent pediatric specialty level at alternate hospitals. While certain alternate hospitals (e.g., Community Medical Centers, Saint Agnes Medical Center, Adventist Health facilities) may provide limited or general versions of some services, they do not offer the same pediatric-focused scope, capacity, or designations as Valley Children's Hospital

9. The Plan's PPO line of business is licensed by the Department. Please confirm the following: **Not applicable**
- a. The number of PPO members who reside within a 15 mile radius of the Terminating Hospital,
 - b. Whether all of the Alternate Hospitals are in-network facilities for PPO members,
 - c. Whether the Plan's PPO providers can admit for and provide all medically necessary services at the Alternate Hospitals,
 - d. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.

For each affected provider group that primarily utilizes the Terminating Hospital, please summarize or confirm the following:

Plan Response: The Plan is in the process of conducting outreach to affected provider groups that primarily utilize Valley Children's Hospital; documentation of contacts (including names, phone numbers, and email addresses) is pending completion and will be provided upon request.

To date, Anthem is assessing provider group feedback, including any expressed concerns, and evaluating the extent to which provider groups have routine access to Valley Children's Hospital.

Anthem has preliminarily confirmed that affected provider groups have the ability to redirect enrollees to alternate in-network hospitals for routine inpatient and emergency services, primarily through existing admitting privileges and/or hospitalist coverage arrangements.

10.

- a. Whether the provider group has been contacted,
- b. Name of contact, phone number, and email address,
- c. Whether the provider group expressed concerns,
- d. Whether the provider group has routine access to the Terminating Hospital,
- e. The ability of the provider group to redirect enrollees to the Alternate Hospitals:
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals, **The Plan is evaluating whether any specialties rely exclusively on admitting privileges at Valley Children's Hospital; this analysis is pending. To the extent such specialties are identified, Anthem will ensure**

access through continuity of care, out-of-network authorization, and/or single-case agreements

1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
 - ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.). **Plan Response: Where provider groups do not maintain direct admitting privileges at alternate hospitals, admissions will occur through established hospitalist coverage or Plan-directed admission processes. Identification of specific hospitalist groups, staffing levels, and capabilities is pending validation; however, Anthem expects that hospitalist arrangements will support admission for medically necessary services in accordance with Medi-Cal requirements.**
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.
11. The Plan included Community Regional Medical Center – Fresno as an Alternate Hospital; however, was not included as an Alternate Hospital for LaSalle Medical Associates. Please confirm whether Community Regional Medical Center – Fresno is an Alternate Hospital for MemorialCare Medical Foundation. If yes, please provide the following: **The Plan confirms CRMC is not an alternate hospital for MemorialCare**
 - a. Number of PCPs and/or Hospitalists who admit to the Alternate Hospital,
 - b. Number of Specialist who admit to the Alternate Hospital,
 - c. Date Contract Terminates/Renews,
 - d. Termination Notice Days,
 - e. Whether a notice of contract termination is sent or received,
 - f. Whether there is a potential cost increase for PPO/POS enrollees, and
 - g. Whether the contract is evergreen and will auto-renew.
12. For each Alternate Hospital, please summarize or confirm the following:
 - a. The Plan contacted the hospital regarding the Block Transfer, **Termination is for 7/1/26- communications with alternate hospitals scheduled for 5/1/26**
 - b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer, **None to date**

- c. Whether the Plan has sent or received a notice of contract termination from the Alternate Hospital, and if so, whether that contract is set to terminate within the next six months. **No**
13. Please confirm whether the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate. **The Plan confirms that HMO providers in the Central Valley have adequate admitting privileges and/or hospitalist coverage arrangements at contracted alternate hospitals for routine inpatient and emergency services, consistent with Medi-Cal network adequacy requirements. However, Anthem does not have an alternate in-network hospital in the region that provides the full scope of pediatric tertiary and quaternary services currently furnished by Valley Children's Hospital. Accordingly, admitting privileges are adequate for general services, but not for all specialized pediatric services that would be lost. Anthem will ensure access to those services through continuity of care, out-of-network authorization, and/or single-case agreements, in compliance with California Medi-Cal requirements.**
14. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals: **Valley Children's Hospital provides a range of pediatric-specific inpatient and outpatient services that are not available, or not available at equivalent scope or designation, at the identified alternate in-network hospitals in the Central Valley.**
 - a. Any out-patient or in-patient services, **Services not fully available at alternate hospitals include comprehensive pediatric subspecialty care, including but not limited to pediatric cardiology, neurology, hematology/oncology, endocrinology, gastroenterology, and other pediatric specialty clinics and coordinated outpatient programs, as well as associated inpatient pediatric specialty care**
 - b. Any tertiary care, quaternary care, cancer, burn or other high-level specialty services, **Services not available at equivalent level include Level IV NICU (regional neonatal intensive care), pediatric intensive care (PICU), pediatric trauma services, extracorporeal life support (ECMO), advanced pediatric epilepsy services, and other pediatric tertiary/quaternary specialty services.** Alternate hospitals in the region do not maintain equivalent pediatric designations or capabilities.
 - c. Any transplant services. **Valley Children's Hospital offers pediatric bone marrow/stem cell transplant and advanced cell therapy services, which are not available at the identified alternate in-network hospitals in the region. Accordingly, while alternate hospitals provide general acute care services, they do not provide equivalent pediatric specialty, tertiary, or transplant services, and Anthem Blue Cross Medi-Cal will ensure access to such services through continuity of care, out-of-network**

authorization, and/or single-case agreements, consistent with California Medi-Cal requirements.

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination. For services that are not available at the identified alternate in-network hospitals—specifically pediatric tertiary and quaternary services such as Level IV NICU, PICU, pediatric trauma, ECMO, comprehensive pediatric subspecialty care, and pediatric bone marrow/stem cell transplant—Anthem Blue Cross Medi-Cal will ensure continued access through the following mechanisms:

Continuity of Care: Members currently receiving treatment at Valley Children's Hospital may continue to receive services there for a period of time, as required under California Medi-Cal continuity-of-care provisions, when medically appropriate.

Out-of-Network Authorization: Anthem will authorize medically necessary services at Valley Children's Hospital or another qualified pediatric specialty facility when equivalent services are not available within the contracted network.

Single-Case Agreements (SCAs): Where appropriate, Anthem may enter into single-case agreements with Valley Children's Hospital or other qualified providers to ensure access to required services.

Referral to Qualified Regional Centers: If needed, members may be referred to other contracted or approved pediatric tertiary/quaternary facilities within California capable of providing the required level of care.

These approaches ensure that affected enrollees continue to receive medically necessary services in a timely manner and in compliance with California Medi-Cal managed care requirements, including network adequacy and access-to-care standards.

15. In the event of contract termination where the Plan removes the Terminating Hospital from its contracted provider network, please summarize or confirm the following that may result: As the anticipated contract termination is not effective until July 1, 2026, Anthem Blue Cross Medi-Cal has not yet finalized the post-termination time and distance analysis.
 - a. The total number of enrollees, if any, who have a residence or workplace beyond 30 minutes or 15 miles of a contracting or plan-operated hospital, The number of enrollees who may reside or work beyond 30 minutes or 15 miles of a contracted hospital is pending validation closer to the termination date.
 - b. The number of enrollees from 15.a. within each corresponding zip code, ZIP code-level impacts are likewise pending completion of that analysis.

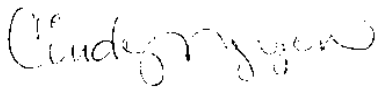
- c. The Plan's process for ensuring compliance with Cal. Code Regs., Tit. 28, § 1300.51(d)(H)(ii). (ii) Hospitals. In the case of a full-service plan, all enrollees have a residence or workplace within 30 minutes or 15 miles of a contracting or plan-operated hospital which has a capacity to serve the entire dependent enrollee population based on normal utilization, and, if separate from such hospital, a contracting or plan-operated provider of all emergency health care services. The Plan will ensure compliance with Cal. Code Regs., Tit. 28, § 1300.51(d)(H)(ii) by conducting an updated geo-access analysis prior to the effective date and implementing any necessary mitigation strategies (e.g., network adjustments or out-of-network arrangements) to maintain required access standards.
16. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital. The Plan confirms that contracted medical groups in the Central Valley that do not utilize Valley Children's Hospital as a primary hospital, but have admitted enrollees there, are able to refer members to other contracted in-network hospitals for routine inpatient, emergency, and general acute care services, consistent with Medi-Cal managed care requirements. However, comparable in-network services are not fully available for certain pediatric tertiary and quaternary services provided by Valley Children's Hospital; accordingly, Anthem will ensure access to those services through continuity of care, out-of-network authorization, and/or single-case agreements with Valley Children's Hospital or other qualified pediatric specialty providers, as medically necessary and in compliance with California Medi-Cal requirements.
17. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination. As of this filing, The Plan is actively validating the number of enrollees who have procedures that are both authorized and scheduled at Valley Children's Hospital for dates following the anticipated contract termination. Anthem will provide this information to DMHC upon completion of its review. In the interim, Anthem will ensure that any identified enrollees are able to proceed with their scheduled procedures without disruption through continuity of care and/or completion of covered services, or, if necessary, through out-of-network authorization or single-case agreements, in accordance with California Medi-Cal managed care requirements.
18. Please indicate whether a continuity of care and/or completion of covered services agreement exists between the Plan and the Terminating Hospital. If no agreement exists, please provide an update on the progress, if any, of reaching an agreement. As of this filing, the Plan does not have a separate, executed continuity of care or completion of covered services agreement with Valley Children's Hospital specific to the contract termination.

19. Please confirm whether the provider group(s) in this Block Transfer Filing are facing potential contract termination(s) within six months. In addition, if the Plan is required to file a Block Transfer Filing for any of the provider group(s), please provide the name of the provider group and the filing number of each Block Transfer Filing. **The Plan confirms that, as of this filing, the provider group(s) included in this Block Transfer Filing are not subject to known or anticipated contract terminations within the next six months.**

Please submit the Plan's response by **March 25, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in cursive script, appearing to read "Cindy Nguyen".

Cindy Nguyen
Analyst II
Office of Plan Monitoring